

kind of standardised clinical protocol sought by Larsen and colleagues could rather generate an (implicitly) inflexible attitude by forcing living phenomena to fit the designed taxonomy, thus recreating the totalising and hegemonic errors of the DSM. We think that a brief word of caution is in order here about the need for an open, pluralistic, and flexible approach.

We declare no competing interests.

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Authors' reply

We thank our esteemed colleagues for welcoming our proposal to initiate and spearhead the development of the Ontology of Phenomenological Psychopathology (OPheP).¹ Rosa Ritunnano and colleagues rightly point out that for the OPheP initiative to maximise its potential, it must avoid the hard-learned pitfalls of the

DSM and ICD projects. These have been criticised for their non-open and centralised approaches, inflexible taxonomies, and contribution to the stagnation of research and clinical practice.² We agree with the authors that moving OPheP in this direction will be counterproductive to integration efforts and antithetical to the tradition of phenomenology.

Rather, the OPheP initiative is committed to decentralised, collaborative, and open-science practices³ that are clearly distinct from the institutionally led and institutionally controlled approaches to categorisation that have governed mainstream psychiatry thus far. Open-source ontologies, including OPheP, evolve through active community participation, which entails embracing pluralism. The editorial work consists of transparently cataloguing areas of consensus and dissent between different experts' perspectives, rather than releasing decisions reached by committees operating behind closed doors. Insofar as OPheP aspires to serve one day as a resource offering an ever-evolving taxonomy of entities and relations in phenomenological psychopathology, it can only achieve this aim by generating obvious use to its community, supporting rather than restricting their work.

OPheP follows in a long tradition of successful ontologies developed via an open, decentralised approach.⁴ For example, the widely-used Gene Ontology⁵ includes thousands of terms that scientists use to describe the functions of genes, for which each term constitutes an evolving expert consensus regarding its proper label, definition, relations, evidence, and so on. When developing OPheP, phenomenologists must set similar collaborative goals as they shape and continuously revise the comprehensive vocabulary that future phenomenologists and psychiatrists can draw from when they attend to and describe first-person experiences

and high-order disturbances of mental health.

We declare no competing interests.

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Mental health care in Uganda

During the opening of mental health month (May, 2022), officials from the Ministry of Health in Uganda stated that about 14 million people of the 43·7 million population have a mental illness. The approximate prevalence of about 32·0% was higher than in previous national estimates of 24·2%;¹ the prevalence might be higher due to the pervasive social stigma and taboos associated with accessing mental health services and the impact of the COVID-19 pandemic. The primary care system in Uganda is poorly resourced, inadequately funded, and ill-equipped to address mental health concerns.² Nationwide, there are only 53 psychiatrists—approximately one psychiatrist per 1 million population.